



ARIZONA HOUSE OF REPRESENTATIVES FLOOR AMENDMENT EXPLANATION

57th Legislature, 2nd Regular Session

Majority Research Staff

SB 1630: home; community-based services; mental illness

BLISS FLOOR AMENDMENT

1. Renames the HCBS Program for SMI adults the *SMI Enhanced Residential Treatment Program* which is a three-year pilot program.
2. Contains enrollment, waitlist management and stakeholder workgroup provisions.
3. Outlines a grievance and appeal process and provider requirements.
4. Defines terms.
5. Makes technical and conforming changes.

BLISS FLOOR AMENDMENT
HOUSE OF REPRESENTATIVES AMENDMENTS TO S.B. 1630
(Reference to Senate engrossed bill)

Amendment instruction key:

[GREEN UNDERLINING IN BRACKETS] indicates text added to statute or previously enacted session law.

[Green underlining in brackets] indicates text added to new session law or text restoring existing law.

[~~GREEN STRIKEOUT IN BRACKETS~~] indicates new text removed from statute or previously enacted session law.

[~~Green strikeout in brackets~~] indicates text removed from existing statute, previously enacted session law or new session law.

<<Green carets>> indicate a section added to the bill.

<<~~Green strikeout in carets~~>> indicates a section removed from the bill.

1 The bill as proposed to be amended is reprinted as follows:

2 <<Section 1. Section 36-401, Arizona Revised Statutes, is amended
3 to read:

4 36-401. Definitions: adult foster care

5 A. In this chapter, unless the context otherwise requires:

6 1. "Accredited health care institution" means a health care
7 institution, other than a hospital, that is currently accredited by a
8 nationally recognized accreditation organization.

9 2. "Accredited hospital" means a hospital that is currently
10 accredited by a nationally recognized organization on hospital
11 accreditation.

12 3. "Adult behavioral health therapeutic home" means a residence for
13 individuals who are at least eighteen years of age, have behavioral health
14 issues and need behavioral health services that does all of the following
15 for those individuals:

16 (a) Provides room and board.

17 (b) Assists in acquiring daily living skills.

18 (c) Coordinates transportation to scheduled appointments.

19 (d) Monitors behaviors.

20 (e) Assists in the self-administration of medication.

21 (f) Provides feedback to case managers related to behavior.

22 4. "Adult day health care facility" means a facility that provides
23 adult day health services during a portion of a continuous
24 twenty-four-hour period for compensation on a regular basis for five or
25 more adults who are not related to the proprietor.

26 5. "Adult day health services" means a program that provides
27 planned care supervision and activities, personal care, personal living
28 skills training, meals and health monitoring in a group setting during a

1 portion of a continuous twenty-four-hour period. Adult day health
2 services may also include preventive, therapeutic and restorative
3 health-related services that do not include behavioral health services.

4 6. "Adult foster care home" means a residential setting that
5 provides room and board and adult foster care services for at least one
6 and not more than four adults who are participants in the Arizona
7 long-term care system pursuant to chapter 29, article 2 of this title or
8 contracts for services with the United States department of veterans
9 affairs and in which the sponsor or the manager resides with the residents
10 and integrates the residents who are receiving adult foster care into that
11 person's family.

12 7. "Adult foster care services" means supervision, assistance with
13 eating, bathing, toileting, dressing, self-medication and other routines
14 of daily living or services authorized by rules adopted pursuant to
15 section 36-405 and section 36-2939, subsection C.

16 8. "Assisted living center" means an assisted living facility that
17 provides resident rooms or residential units to eleven or more residents.

18 9. "Assisted living facility" means a residential care institution,
19 including an adult foster care home, that provides or contracts to provide
20 supervisory care services, personal care services or directed care
21 services on a continuous basis.

22 10. "Assisted living home" means an assisted living facility that
23 provides resident rooms to ten or fewer residents.

24 11. "Behavioral health services" means services that pertain to
25 mental health and substance use disorders and that are either:

26 (a) Performed by or under the supervision of a professional who is
27 licensed pursuant to title 32 and whose scope of practice allows the
28 professional to provide these services.

29 (b) Performed on behalf of patients by behavioral health staff as
30 prescribed by rule.

31 12. "Construction" means building, erecting, fabricating or
32 installing a health care institution.

33 13. "Continuous" means available at all times without cessation,
34 break or interruption.

35 14. "Controlling person" means a person who:

36 (a) Through ownership, has the power to vote at least ten percent
37 of the outstanding voting securities.

38 (b) If the applicant or licensee is a partnership, is the general
39 partner or a limited partner who holds at least ten percent of the voting
40 rights of the partnership.

41 (c) If the applicant or licensee is a corporation, an association
42 or a limited liability company, is the president, the chief executive
43 officer, the incorporator or any person who owns or controls at least ten
44 percent of the voting securities. For the purposes of this subdivision,
45 corporation does not include nonprofit corporations.

1 (d) Holds a beneficial interest in ten percent or more of the
2 liabilities of the applicant or the licensee.

3 15. "Department" means the department of health services.

4 16. "Directed care services" means programs and services, including
5 supervisory and personal care services, that are provided to persons who
6 are incapable of recognizing danger, summoning assistance, expressing need
7 or making basic care decisions.

8 17. "Direction" means authoritative policy or procedural guidance
9 to accomplish a function or activity.

10 18. "Director" means the director of the department.

11 19. "Direct owner" means a person that has an ownership or control
12 interest in a health care institution totaling fifty-one percent or more.

13 [20. "ENHANCED RESIDENTIAL TREATMENT FACILITY" MEANS A RESIDENTIAL
14 CARE INSTITUTION THAT IS LICENSED TO PROVIDE PROGRAMS AND SERVICES,
15 INCLUDING BEHAVIORAL HEALTH SERVICES, STRUCTURED SUPPORT SERVICES AND
16 HEALTH-RELATED SERVICES, TO RESIDENTS WHO ARE SERIOUSLY MENTALLY ILL AS
17 DEFINED IN SECTION 36-550.]

18 ~~[20.]~~ [21.] "Facilities" means buildings that are used by a health
19 care institution for providing any of the types of services as defined in
20 this chapter.

21 ~~[21.]~~ [22.] "Freestanding urgent care center":

22 (a) Means an outpatient treatment center that, regardless of its
23 posted or advertised name, meets any of the following requirements:

24 (i) Is open twenty-four hours a day, excluding at its option
25 weekends or certain holidays, but is not licensed as a hospital.

26 (ii) Claims to provide unscheduled medical services that are not
27 otherwise routinely available in primary care physician offices.

28 (iii) By its posted or advertised name, gives the impression to the
29 public that it provides medical care for urgent, immediate or emergency
30 conditions.

31 (iv) Routinely provides ongoing unscheduled medical services for
32 more than eight consecutive hours for an individual patient.

33 (b) Does not include the following:

34 (i) A medical facility that is licensed under a hospital's license
35 and that uses the hospital's medical provider number.

36 (ii) A qualifying community health center pursuant to section
37 36-2907.06.

38 (iii) Any other health care institution licensed pursuant to this
39 chapter.

40 (iv) A physician's office that offers extended hours or same-day
41 appointments to existing and new patients and that does not meet the
42 requirements of subdivision (a), item (i), (iii) or (iv) of this
43 paragraph.

44 ~~[22.]~~ [23.] "Governing authority" means the individual, agency,
45 partners, group or corporation, whether appointed, elected or otherwise

1 designated, in which the ultimate responsibility and authority for the
2 conduct of the health care institution are vested.

3 ~~[23.]~~ [24.] "Health care institution" means every place,
4 institution, building or agency, whether organized for profit or not, that
5 provides facilities with medical services, nursing services, behavioral
6 health services, health screening services, other health-related services,
7 supervisory care services, personal care services or directed care
8 services and includes home health agencies as defined in section 36-151,
9 outdoor behavioral health care programs and hospice service agencies.

10 ~~[24.]~~ [25.] "Health-related services" means services, other than
11 medical, that pertain to general supervision, protective, preventive and
12 personal care services, supervisory care services or directed care
13 services.

14 ~~[25.]~~ [26.] "Health screening services" means the acquisition,
15 analysis and delivery of health-related data of individuals to aid in
16 determining the need for medical services.

17 ~~[26.]~~ [27.] "Hospice" means a hospice service agency or the
18 provision of hospice services in an inpatient facility.

19 ~~[27.]~~ [28.] "Hospice service" means a program of palliative and
20 supportive care for terminally ill persons and their families or
21 caregivers.

22 ~~[28.]~~ [29.] "Hospice service agency" means an agency or
23 organization, or a subdivision of that agency or organization, that
24 provides hospice services at the place of residence of its clients.

25 ~~[29.]~~ [30.] "Indirect owner":

26 (a) Means a person that has an ownership or control interest in a
27 direct owner totaling fifty-one percent or more.

28 (b) Includes an ownership or control interest in an indirect owner
29 totaling fifty-one percent or more and a combination of direct ownership
30 and indirect ownership or control interests totaling fifty-one percent or
31 more in the health care institution.

32 ~~[30.]~~ [31.] "Inpatient beds" or "resident beds" means
33 accommodations with supporting services, such as food, laundry and
34 housekeeping, for patients or residents who generally stay in excess of
35 twenty-four hours.

36 ~~[31.]~~ [32.] "Intermediate care facility for individuals with
37 intellectual disabilities" has the same meaning prescribed in section
38 36-551.

39 ~~[32.]~~ [33.] "Licensed capacity" means the total number of persons
40 for whom the health care institution is authorized by the department to
41 provide services as required pursuant to this chapter if the person is
42 expected to stay in the health care institution for more than twenty-four
43 hours. For a hospital, licensed capacity means only those beds specified
44 on the hospital license.

45 ~~[33.]~~ [34.] "Medical services" means the services that pertain to
46 medical care and that are performed at the direction of a physician on

1 behalf of patients by physicians, dentists, nurses and other professional
2 and technical personnel.

3 ~~[34.]~~ [35.] "Modification" means the substantial improvement,
4 enlargement, reduction or alteration of or other change in a health care
5 institution.

6 ~~[35.]~~ [36.] "Nonproprietary institution" means any health care
7 institution that is organized and operated exclusively for charitable
8 purposes, no part of the net earnings of which inures to the benefit of
9 any private shareholder or individual, or that is operated by the state or
10 any political subdivision of the state.

11 ~~[36.]~~ [37.] "Nursing care institution" means a health care
12 institution that provides inpatient beds or resident beds and nursing
13 services to persons who need continuous nursing services but who do not
14 require hospital care or direct daily care from a physician.

15 ~~[37.]~~ [38.] "Nursing services" means those services that pertain
16 to the curative, restorative and preventive aspects of nursing care and
17 that are performed at the direction of a physician by or under the
18 supervision of a registered nurse licensed in this state.

19 ~~[38.]~~ [39.] "Nursing-supported group home" means a health care
20 institution that is a community residential setting as defined in section
21 36-551 for not more than six persons with developmental disabilities, that
22 is operated by a service provider under contract with the department of
23 economic security and that provides room and board, daily habilitation and
24 continuous nursing support and intervention.

25 ~~[39.]~~ [40.] "Organized medical staff" means a formal organization
26 of physicians, and dentists if appropriate, with the delegated authority
27 and responsibility to maintain proper standards of medical care and to
28 plan for continued betterment of that care.

29 ~~[40.]~~ [41.] "Outdoor behavioral health care program" means an
30 agency that provides behavioral health services in an outdoor environment
31 as an alternative to behavioral health services that are provided in a
32 health care institution with facilities. Outdoor behavioral health care
33 programs do not include:

34 (a) Programs, facilities or activities that are operated by a
35 government entity or that are licensed by the department as a child care
36 program pursuant to chapter 7.1 of this title.

37 (b) Outdoor activities for youth that are designated to be
38 primarily recreational and that are organized by church groups, scouting
39 organizations or similar groups.

40 (c) Outdoor youth programs that are licensed by the department of
41 economic security.

42 ~~[41.]~~ [42.] "Personal care services" means assistance with
43 activities of daily living that can be performed by persons without
44 professional skills or professional training and includes the coordination
45 or provision of intermittent nursing services and the administration of

1 medications and treatments by a nurse who is licensed pursuant to title
2 32, chapter 15 or as otherwise provided by law.

3 ~~[42.]~~ [43.] "Physician" means any person who is licensed pursuant
4 to title 32, chapter 13 or 17.

5 ~~[43.]~~ [44.] "Recidivism reduction services" means services that
6 are delivered by an adult residential care institution to its residents to
7 encourage lawful behavior and to discourage or prevent residents who are
8 suspected of, charged with or convicted of one or more criminal offenses,
9 or whose mental health and substance use can be reasonably expected to
10 place them at risk for the future threat of prosecution, diversion or
11 incarceration, from engaging in future unlawful behavior.

12 ~~[44.]~~ [45.] "Recidivism reduction staff" means a person who
13 provides recidivism reduction services.

14 ~~[45.]~~ [46.] "Residential care institution" means a health care
15 institution other than a hospital or a nursing care institution that
16 provides resident beds or residential units, supervisory care services,
17 personal care services, behavioral health services, directed care services
18 or health-related services for persons who do not need continuous nursing
19 services.

20 ~~[46.]~~ [47.] "Residential unit" means a private apartment, unless
21 otherwise requested by a resident, that includes a living and sleeping
22 space, kitchen area, private bathroom and storage area.

23 ~~[47.]~~ [48.] "Respite care services" means services that are
24 provided by a licensed health care institution to persons who are
25 otherwise cared for in foster homes and in private homes to provide an
26 interval of rest or relief of not more than thirty days to operators of
27 foster homes or to family members.

28 [49. "STRUCTURED SUPPORT SERVICES" INCLUDES SERVICES THAT ARE
29 AVAILABLE FOR UP TO TWENTY-FOUR-HOURS PER DAY AND CASE MANAGEMENT, CRISIS
30 INTERVENTION, SOCIAL SKILLS TRAINING AND BUDGETING ASSISTANCE.]

31 ~~[48.]~~ [50.] "Substantial compliance" means that the nature or
32 number of violations revealed by any type of inspection or investigation
33 of a health care institution does not pose a direct risk to the life,
34 health or safety of patients or residents.

35 ~~[49.]~~ [51.] "Supervision" means directly overseeing and inspecting
36 the act of accomplishing a function or activity.

37 ~~[50.]~~ [52.] "Supervisory care services" means general supervision,
38 including daily awareness of resident functioning and continuing needs,
39 the ability to intervene in a crisis and assistance in self-administering
40 prescribed medications.

41 ~~[51.]~~ [53.] "Temporary license" means a license that is issued by
42 the department to operate a class or subclass of a health care institution
43 at a specific location and that is valid until an initial licensing
44 inspection.

45 ~~[52.]~~ [54.] "Unscheduled medical services" means medically
46 necessary periodic health care services that are unanticipated or cannot

1 reasonably be anticipated and that require medical evaluation or treatment
2 before the next business day.

3 B. If there are fewer than four Arizona long-term care system
4 participants receiving adult foster care in an adult foster care home,
5 nonparticipating adults may receive other types of services that are
6 authorized by law to be provided in the adult foster care home as long as
7 the number of adults served, including the Arizona long-term care system
8 participants, does not exceed four.

9 C. Nursing care services may be provided by the adult foster care
10 licensee if the licensee is a nurse who is licensed pursuant to title 32,
11 chapter 15 and the services are limited to those allowed pursuant to law.
12 The licensee shall keep a record of nursing services rendered. >>

13 <<Sec. 2. Title 36, chapter 4, article 2, Arizona Revised Statutes,
14 is amended by adding section 36-425.10, to read:

15 36-425.10. Enhanced residential treatment facilities;
16 licensure; rules

17 [THE DEPARTMENT SHALL LICENSE ENHANCED RESIDENTIAL TREATMENT
18 FACILITIES CONSISTENT WITH THIS CHAPTER. THE DEPARTMENT SHALL ADOPT RULES
19 FOR ENHANCED RESIDENTIAL TREATMENT FACILITIES, INCLUDING LICENSURE
20 STANDARDS, STAFFING REQUIREMENTS AND PHYSICAL PLANT STANDARDS. AN
21 ENHANCED RESIDENTIAL TREATMENT FACILITY MAY PROVIDE PROGRAMS AND SERVICES,
22 THROUGH A COMBINATION OF FACILITY-BASED OR COORDINATED COMMUNITY-BASED
23 SERVICES, TO RESIDENTS WHO ARE SERIOUSLY MENTALLY ILL AS DEFINED IN
24 SECTION 36-550.]>>

25 <<Sec. 3. Section 36-2935, Arizona Revised Statutes, is amended to
26 read:

27 36-2935. Estate recovery program; liens

28 A. The director shall adopt rules in accordance with state and
29 federal law to allow the administration to file a claim against a member's
30 estate to recover paid assistance. The administration is also entitled to
31 a lien on a member's property to recover paid assistance the member
32 receives.

33 B. A member's personal representative must notify the
34 administration of the member's estate or property within three months
35 after the member's death if the member was at least fifty-five years of
36 age and the administration has not already filed a statement of claim in
37 the estate proceedings.

38 C. As nearly as is possible, the administration shall recover
39 charges pursuant to the procedures prescribed in sections 36-2915 and
40 36-2916. If both the administration and a county have valid liens for
41 paid assistance provided to the same member, or if both the administration
42 and a special health care district have valid claims for paid assistance
43 provided to the same member, the value of the property shall be divided
44 between the administration, the special health care district and the
45 county pro rata according to the amounts of their respective liens.

1 D. The administration shall impose liens in a manner consistent
2 with federal law.

3 E. This section also applies to[:

4 1.] Persons who are eligible pursuant to section 36-2901, paragraph
5 6, subdivision (a) and who receive medical assistance under article 1 of
6 this chapter.

7 [2. PERSONS WHO ARE ELIGIBLE FOR THE SERIOUSLY MENTALLY ILL
8 ENHANCED RESIDENTIAL TREATMENT PILOT PROGRAM PURSUANT TO ARTICLE 3.1 OF
9 THIS CHAPTER.]>>

10 Sec. 4. Title 36, chapter 29, Arizona Revised Statutes, is amended
11 by adding article 3.1, to read:

12 ARTICLE 3.1. [HOME AND COMMUNITY-BASED SERVICES
13 FOR PERSONS WITH SERIOUS MENTAL ILLNESS] [SERIOUSLY MENTALLY ILL
14 ENHANCED RESIDENTIAL TREATMENT PILOT PROGRAM]

15 36-2979. Definitions

16 IN THIS ARTICLE, UNLESS THE CONTEXT OTHERWISE REQUIRES:

17 1. "ADMINISTRATION" MEANS THE ARIZONA HEALTH CARE COST CONTAINMENT
18 SYSTEM ADMINISTRATION.

19 [2. "BEHAVIORAL HEALTH RESIDENTIAL FACILITY" MEANS A HEALTH CARE
20 INSTITUTION THAT IS LICENSED PURSUANT TO THIS TITLE TO PROVIDE LEVEL I OR
21 II BEHAVIORAL HEALTH RESIDENTIAL SERVICES.

22 3. "HIGH-ACUITY SERIOUSLY MENTALLY ILL INDIVIDUAL" MEANS A PERSON
23 WHO MEETS A SERIOUS MENTAL ILLNESS LONG-TERM LEVEL OF CARE AND ONE OR MORE
24 PRIORITY CRITERIA LISTED IN SECTION 36-2979.04, SUBSECTION B.

25 4. "HOME AND COMMUNITY-BASED SERVICES":

26 (a) MEANS SERVICES AUTHORIZED UNDER ONE OR MORE MEDICAID STATE PLAN
27 AUTHORITIES, WAIVERS OR DEMONSTRATION AUTHORITIES, INCLUDING THOSE
28 AUTHORIZED UNDER 42 UNITED STATES CODE SECTION 1315 OR 1396n, THAT SUPPORT
29 INDIVIDUALS IN THE COMMUNITY AS AN ALTERNATIVE TO INSTITUTIONALIZATION.

30 (b) INCLUDES SERVICES THAT PROVIDE ASSISTANCE WITH ACTIVITIES OF
31 DAILY LIVING, MEDICATION ADMINISTRATION, SUPERVISION AND STRUCTURED
32 SUPPORTS COMPARABLE TO ASSISTED LIVING MODELS.]

33 [2. "APPLICANT" MEANS A PERSON WHO APPLIES FOR ENROLLMENT IN THE
34 PILOT PROGRAM.

35 3. "BEHAVIORAL HEALTH SERVICES" HAS THE SAME MEANING PRESCRIBED IN
36 SECTION 36-401.

37 4. "CONTRACTOR" MEANS AN ENTITY THAT HAS ENTERED INTO AN AGREEMENT
38 WITH THE ADMINISTRATION TO PROVIDE ACUTE CARE AND BEHAVIORAL HEALTH
39 SERVICES TO SERIOUSLY MENTALLY ILL INDIVIDUALS PURSUANT TO ARTICLE 1 OF
40 THIS CHAPTER AND CHAPTER 34, ARTICLE 1 OF THIS TITLE.

41 5. "ELIGIBLE INDIVIDUAL" MEANS A PERSON WHO MEETS THE ELIGIBILITY
42 CRITERIA PRESCRIBED IN SECTION 36-2979.02.

43 6. "ENHANCED RESIDENTIAL TREATMENT FACILITY" HAS THE SAME MEANING
44 PRESCRIBED IN SECTION 36-401.

45 7. "ENHANCED RESIDENTIAL TREATMENT SERVICES" MEANS SERVICES
46 AUTHORIZED UNDER FEDERAL MEDICAID AUTHORITY PURSUANT TO SECTION 36-2979.09

1 THAT SUPPORT INDIVIDUALS IN AN ENHANCED RESIDENTIAL TREATMENT FACILITY AND
2 THAT INCLUDE CONTINUOUS SUPERVISION, STRUCTURED SUPPORT SERVICES,
3 MEDICATION ADMINISTRATION AND MONITORING, TREATMENT PLANNING AND
4 COORDINATION AND PERSONAL CARE SERVICES NECESSARY FOR HEALTH AND SAFETY.]
5 [5.] [8.] "MEMBER" MEANS [A PERSON] [AN ELIGIBLE INDIVIDUAL] WHO
6 IS ENROLLED IN THE [PILOT] PROGRAM.
7 [6.] [9.] "[PILOT] PROGRAM" MEANS THE [HOME AND COMMUNITY-BASED
8 SERVICE PROGRAM FOR ADULTS WHO ARE] SERIOUSLY MENTALLY ILL [ENHANCED
9 RESIDENTIAL TREATMENT PILOT PROGRAM ESTABLISHED BY SECTION 36-2979.01].
10 [7. "QUALIFIED INCOME TRUST" MEANS A TRUST AS DESCRIBED IN 42
11 UNITED STATES CODE SECTION 1396p(d)(4)(B).]
12 [8.] [10.] "SERIOUSLY MENTALLY ILL" [OR "SMI"] HAS THE [SAME]
13 MEANING PRESCRIBED IN SECTION 36-550.
14 [9. "SERIOUS MENTAL ILLNESS LONG-TERM LEVEL OF CARE" MEANS THE
15 BEHAVIORAL HEALTH INSTITUTIONAL LEVEL OF CARE ADOPTED BY THE
16 ADMINISTRATION PURSUANT TO SECTION 36-2979.02.]
17 [11. "SMI ENHANCED LEVEL OF CARE" MEANS THE FUNCTIONAL AND
18 BEHAVIORAL LEVEL OF CARE CRITERIA ADOPTED BY THE ADMINISTRATION PURSUANT
19 TO SECTION 36-2979.02.
20 12. "STRUCTURED SUPPORT SERVICES" HAS THE SAME MEANING PRESCRIBED
21 IN SECTION 36-401.]
22 36-2979.01. Seriously mentally ill enhanced residential
23 treatment pilot program; stakeholder work
24 group; semiannual implementation updates;
25 enrollment cap; federal approval
26 [A. THE HOME AND COMMUNITY-BASED SERVICE PROGRAM FOR ADULTS WHO ARE
27 SERIOUSLY MENTALLY ILL IS ESTABLISHED. THE ADMINISTRATION SHALL DEVELOP
28 AND REQUEST AUTHORITY FROM THE CENTERS FOR MEDICARE AND MEDICAID SERVICES
29 TO IMPLEMENT A DEDICATED HOME AND COMMUNITY-BASED SERVICES BENEFIT FOR
30 ADULTS WHO ARE DETERMINED TO BE SERIOUSLY MENTALLY ILL.]
31 [A. SUBJECT TO THE APPROVAL OF THE CENTERS FOR MEDICARE AND
32 MEDICAID SERVICES, AVAILABLE APPROPRIATIONS AND THE ENROLLMENT CAP
33 PRESCRIBED IN SUBSECTION F OF THIS SECTION, THE SERIOUSLY MENTALLY ILL
34 ENHANCED RESIDENTIAL TREATMENT PILOT PROGRAM IS ESTABLISHED FOR THREE
35 YEARS FROM THE DATE THAT THE CENTERS FOR MEDICARE AND MEDICAID SERVICES
36 APPROVE THE PILOT PROGRAM CONSISTENT WITH THIS ARTICLE. THE PILOT PROGRAM
37 OPERATES THROUGH AND IS ADMINISTERED BY CONTRACTORS PURSUANT TO THEIR
38 AGREEMENTS WITH THE ADMINISTRATION UNDER CHAPTER 34, ARTICLE 1 OF THIS
39 TITLE. THE ADMINISTRATION SHALL SEEK ANY NECESSARY FEDERAL AUTHORITY TO
40 IMPLEMENT THE PILOT PROGRAM, INCLUDING AUTHORITY UNDER 42 UNITED STATES
41 CODE SECTION 1315 OR OTHER APPLICABLE FEDERAL AUTHORITY, AND SHALL REQUEST
42 APPROVAL NOT LATER THAN JULY 1, 2027. THE ADMINISTRATION SHALL BEGIN
43 ENROLLING ELIGIBLE INDIVIDUALS NOT LATER THAN ONE YEAR AFTER RECEIVING
44 FEDERAL APPROVAL, SUBJECT TO THE AVAILABLE APPROPRIATIONS.
45 B. THE ADMINISTRATION SHALL INCORPORATE THE REQUIREMENTS OF THIS
46 ARTICLE INTO CONTRACTOR AGREEMENTS EXECUTED PURSUANT TO SECTIONS 36-3410

1 AND 36-3412 AND SHALL REQUIRE CONTRACTORS TO FULFILL THE OBLIGATIONS OF
2 THIS ARTICLE AS PART OF THEIR CONTRACTUAL DUTIES.]

3 ~~[B.]~~ [C.] THROUGH THE SUBMISSION OF THE FIRST ANNUAL REPORT
4 [REQUIRED BY SECTION 36-2979.08], THE ADMINISTRATION SHALL CONVENE A
5 STAKEHOLDER [WORKGROUP] [WORK GROUP] THAT INCLUDES [REPRESENTATIVES FROM]
6 [INDIVIDUALS WHO ARE SERIOUSLY MENTALLY ILL AND] BEHAVIORAL HEALTH
7 PROVIDERS, FAMILY MEMBERS, CAREGIVERS AND GUARDIANS OF INDIVIDUALS WHO ARE
8 SERIOUSLY MENTALLY ILL, COUNTIES, TRIBAL NATIONS AND COMMUNITY
9 ORGANIZATIONS TO ADVISE ON [PILOT] PROGRAM DESIGN AND
10 IMPLEMENTATION. STAKEHOLDER INPUT SHALL SPECIFICALLY ADDRESS THE NEEDS OF
11 INDIVIDUALS WHO REQUIRE COMPLEX MEDICATION MANAGEMENT, ENHANCED MONITORING
12 AND STRUCTURED [COMMUNITY-BASED SUPPORTS, INCLUDING ASSISTED LIVING-TYPE
13 MODELS,] [SUPPORT SERVICES] TO PROMOTE MEDICATION CONTINUITY AND SAFETY.

14 ~~[C. THE ADMINISTRATION SHALL REQUEST APPROVAL FROM THE CENTERS FOR~~
15 ~~MEDICARE AND MEDICAID SERVICES FOR THE PROGRAM NOT LATER THAN JULY 1, 2027~~
16 ~~AND SHALL BEGIN IMPLEMENTING THE PROGRAM NOT LATER THAN ONE YEAR AFTER THE~~
17 ~~APPROVAL BY THE CENTERS FOR MEDICARE AND MEDICAID SERVICES.]~~

18 D. UNTIL THE [PILOT] PROGRAM IS IMPLEMENTED, THE ADMINISTRATION
19 SHALL PROVIDE SEMIANNUAL IMPLEMENTATION UPDATES TO THE PRESIDENT OF THE
20 SENATE, THE SPEAKER OF THE HOUSE OF REPRESENTATIVES AND THE CHAIRPERSONS
21 OF THE SENATE AND HOUSE OF REPRESENTATIVES HEALTH AND HUMAN SERVICES
22 COMMITTEES, OR THEIR SUCCESSOR COMMITTEES.

23 [E. THE ADMINISTRATION MAY IMPLEMENT THIS ARTICLE THROUGH POLICY,
24 CONTRACT REQUIREMENTS AND OTHER ADMINISTRATIVE MECHANISMS AUTHORIZED UNDER
25 SECTIONS 36-2903, 36-2904 AND 36-3412.

26 F. SUBJECT TO THE APPROVAL OF THE CENTERS FOR MEDICARE AND MEDICAID
27 SERVICES AND TO AVAILABLE APPROPRIATIONS, THE ADMINISTRATION SHALL
28 IMPLEMENT THE PILOT PROGRAM FOR UP TO SIXTY MEMBERS STATEWIDE. THE
29 ADMINISTRATION MAY INCREASE THE ENROLLMENT CAP ABOVE SIXTY MEMBERS DURING
30 THE PILOT PROGRAM IF THE ADMINISTRATION DEMONSTRATES, THROUGH DATA
31 REPORTED PURSUANT TO SECTION 36-2979.08, REDUCED UTILIZATION OF HIGH-COST
32 SERVICES OR COST AVOIDANCE IN STATE-FUNDED SYSTEMS THAT DEMONSTRATES THAT
33 THE PILOT PROGRAM APPROPRIATION COULD SUSTAIN MEMBERS BEYOND THE
34 ENROLLMENT CAP, SUBJECT TO AVAILABLE APPROPRIATIONS AND REVIEW BY THE
35 JOINT LEGISLATIVE BUDGET COMMITTEE.

36 G. A COURT MAY RECOMMEND PARTICIPATION IN THE PILOT PROGRAM BUT MAY
37 NOT COMPEL THE ADMINISTRATION TO EXCEED THE ENROLLMENT CAP SPECIFIED IN
38 SUBSECTION F OF THIS SECTION.]

39 36-2979.02. Eligibility; financial eligibility; SMI enhanced
40 level of care; criteria

41 A. [SUBJECT TO THE APPROVAL OF THE CENTERS FOR MEDICARE AND
42 MEDICAID SERVICES, AVAILABLE MONIES AND THE ENROLLMENT CAP PRESCRIBED IN
43 SECTION 36-2979.01, SUBSECTION F.] TO BE ELIGIBLE FOR THE [PILOT] PROGRAM,
44 AN INDIVIDUAL MUST MEET ALL OF THE FOLLOWING [CRITERIA]:

- 45 [1. BE EIGHTEEN YEARS OF AGE OR OLDER.
46 2. BE A RESIDENT OF THIS STATE.]

1 ~~[1.]~~ [3.] HAVE BEEN DETERMINED TO BE SERIOUSLY MENTALLY ILL.

2 ~~[2.]~~ [4.] MEET THE ~~[SERIOUS MENTAL ILLNESS LONG-TERM]~~ [SMI
3 ENHANCED] LEVEL OF CARE ADOPTED BY THE ADMINISTRATION PURSUANT TO
4 SUBSECTION [c] [E] OF THIS SECTION[, AS DETERMINED PURSUANT TO SECTION
5 36-2979.03].

6 ~~[3. REQUIRE HOME AND COMMUNITY-BASED SERVICES IN ORDER TO AVOID~~
7 ~~PLACEMENT IN A BEHAVIORAL HEALTH RESIDENTIAL FACILITY OR PSYCHIATRIC~~
8 ~~INSTITUTION.~~

9 ~~4.]~~ [5.] MEET THE FINANCIAL ELIGIBILITY REQUIREMENTS PURSUANT TO
10 SUBSECTION B OF THIS SECTION[, AS DETERMINED BY THE ADMINISTRATION].

11 B. [THE ADMINISTRATION SHALL DETERMINE FINANCIAL ELIGIBILITY FOR
12 THE PILOT PROGRAM AND MAY NOT DELEGATE THAT FUNCTION TO A CONTRACTOR.] AN
13 INDIVIDUAL [WHO MEETS THE SERIOUS MENTAL ILLNESS LONG-TERM LEVEL OF CARE
14 ADOPTED BY THE ADMINISTRATION PURSUANT TO SUBSECTION C OF THIS SECTION] IS
15 FINANCIALLY ELIGIBLE FOR THE [PILOT] PROGRAM IF THE INDIVIDUAL'S INCOME
16 DOES NOT EXCEED THREE HUNDRED PERCENT OF THE FEDERAL SUPPLEMENTAL SECURITY
17 INCOME BENEFIT RATE [AND THE INDIVIDUAL MEETS THE RESOURCE REQUIREMENT
18 FOR LONG-TERM CARE PROGRAMS, AS PRESCRIBED BY 42 UNITED STATES CODE
19 SECTION 1396p].

20 [C.] AN INDIVIDUAL WHOSE INCOME EXCEEDS THIS STANDARD MAY ESTABLISH
21 A QUALIFIED TRUST TO ACHIEVE [FINANCIAL] ELIGIBILITY[, PURSUANT TO SECTION
22 36-2934.01 AND SECTION 1917(d)(4)(A), (B) AND (C) OF THE SOCIAL SECURITY
23 ACT]. [RESOURCE AND SPOUSAL IMPOVERISHMENT STANDARDS SHALL BE THE SAME AS
24 REQUIRED PURSUANT TO ARTICLE 2 OF THIS CHAPTER FOR ENROLLEES IN THE
25 ARIZONA LONG-TERM CARE SYSTEM HOME AND COMMUNITY-BASED SERVICES.] [FOR
26 THE PURPOSES OF THIS ARTICLE, THE FOLLOWING SECTIONS OF ARTICLE 2 OF THIS
27 CHAPTER APPLY TO APPLICANTS AND MEMBERS IN THE SAME MANNER THAT THE
28 SECTIONS APPLY TO APPLICANTS AND MEMBERS IN THE ARIZONA LONG-TERM CARE
29 SYSTEM:

30 1. SECTION 36-2934.02 RELATING TO FINANCIAL INSTRUMENTS.

31 2. SECTION 36-2935 RELATING TO ESTATE RECOVERY AND LIENS.

32 D. TO THE EXTENT THAT ANY PROVISION OF ARTICLE 2 OF THIS CHAPTER AS
33 INCORPORATED BY REFERENCE IN SUBSECTION C OF THIS SECTION REFERENCES THE
34 ARIZONA LONG-TERM CARE SYSTEM, THAT REFERENCE SHALL BE CONSTRUED TO APPLY
35 TO THE PILOT PROGRAM ESTABLISHED BY THIS ARTICLE.]

36 ~~[c.]~~ [E.] THE ADMINISTRATION SHALL ADOPT ~~[A SERIOUS MENTAL ILLNESS~~
37 ~~LONG-TERM]~~ [SMI ENHANCED] LEVEL OF CARE SPECIFIC TO INDIVIDUALS WHO ARE
38 SERIOUSLY MENTALLY ILL~~[.]~~ [THAT IS] [THE CRITERIA SHALL BE] BASED ON
39 BEHAVIORAL, FUNCTIONAL AND SAFETY CRITERIA~~[, WHICH]~~ [AND MAY NOT REQUIRE
40 PHYSICAL DISABILITY OR PHYSICAL IMPAIRMENT CRITERIA. THE CRITERIA] MAY
41 INCLUDE ANY OF THE FOLLOWING:

42 1. CURRENT OR RECENT COURT-ORDERED TREATMENT.

43 2. LEGAL GUARDIANSHIP DUE TO PSYCHIATRIC INCAPACITY.

44 3. RECENT DISCHARGE FROM A JAIL OR PRISON, THE STATE HOSPITAL OR A
45 BEHAVIORAL HEALTH RESIDENTIAL FACILITY.

46 4. REPEATED PSYCHIATRIC HOSPITALIZATIONS OR CRISIS EPISODES.

1 5. IMPAIRED JUDGMENT, DISORGANIZATION OR INABILITY TO PERFORM
2 ACTIVITIES OF DAILY LIVING DUE TO PSYCHIATRIC SYMPTOMS.

3 6. DOCUMENTED SAFETY RISKS, INCLUDING ELOPEMENT, ~~[FIRE OR WATER~~
4 ~~MISUSE,]~~ AGGRESSION, DELUSIONAL BEHAVIOR~~[, FIRE OR WATER MISUSE]~~ OR
5 EXPLOITATION RISK.

6 7. HOMELESSNESS OR IMMINENT RISK OF HOMELESSNESS.

7 8. HIGH-INTENSITY OR COMPLEX PSYCHOTROPIC MEDICATION REGIMENS
8 REQUIRING ENHANCED MONITORING TO ENSURE ADHERENCE AND TO IDENTIFY
9 POTENTIAL ADVERSE EFFECTS.

10 9. CLINICAL NEEDS THAT NECESSITATE STRUCTURED ~~[COMMUNITY-BASED~~
11 ~~SUPPORTS, INCLUDING ASSISTED LIVING-TYPE SUPPORTS,]~~ [SUPPORT SERVICES] TO
12 MAINTAIN CONTINUITY OF MEDICATION THERAPY, SUCH AS THOSE AT ELEVATED RISK
13 OF RELAPSE, DECOMPENSATION OR HOSPITALIZATION RELATED TO MEDICATION
14 NONADHERENCE.

15 ~~[D. THE SERIOUS MENTAL ILLNESS LONG-TERM LEVEL OF CARE ADOPTED~~
16 ~~PURSUANT TO SUBSECTION C OF THIS SECTION MAY NOT REQUIRE A NURSING~~
17 ~~FACILITY LEVEL OF CARE OR PHYSICAL DISABILITY OR PHYSICAL IMPAIRMENT~~
18 ~~CRITERIA AND MAY NOT USE A PREADMISSION SCREENING TOOL.~~

19 ~~E.] [E.] THE ADMINISTRATION MAY [REFINE THE ASSESSMENT CRITERIA~~
20 ~~AND] [MODIFY THE ELIGIBILITY] PROCESSES THROUGH [RULE OR] POLICY,~~
21 ~~CONSISTENT WITH [LEGISLATIVE INTENT, INCLUDING ADJUSTMENT TO ASSESSMENT~~
22 ~~TOOLS, THRESHOLDS OR PROCESSES] [THIS ARTICLE, SUBJECT TO ANY NECESSARY~~
23 ~~APPROVAL BY THE CENTERS FOR MEDICARE AND MEDICAID SERVICES AND AVAILABLE~~
24 ~~APPROPRIATIONS].~~

25 ~~[36-2979.03. Covered services; modification~~

26 ~~A. SUBJECT TO THE APPROVAL OF THE CENTERS FOR MEDICARE AND MEDICAID~~
27 ~~SERVICES, THE ADMINISTRATION SHALL PROVIDE HOME AND COMMUNITY-BASED~~
28 ~~SERVICES THAT ARE COMPARABLE TO THOSE AUTHORIZED UNDER ARTICLE 2 OF THIS~~
29 ~~CHAPTER, WITH MODIFICATIONS APPROPRIATE FOR BEHAVIORAL HEALTH NEEDS. THE~~
30 ~~HOME AND COMMUNITY-BASED SERVICES SHALL INCLUDE:~~

31 ~~1. ATTENDANT CARE AND PERSONAL CARE.~~

32 ~~2. HABILITATION WITH BEHAVIOR MANAGEMENT.~~

33 ~~3. ADULT DAY HEALTH CARE WITH A BEHAVIORAL HEALTH FOCUS.~~

34 ~~4. SUPERVISED COMMUNITY LIVING SERVICES, INCLUDING ASSISTED~~
35 ~~LIVING-TYPE SUPPORTS.~~

36 ~~5. RESPITE CARE.~~

37 ~~6. HOME-DELIVERED MEALS.~~

38 ~~7. NURSING, HOME HEALTH AND MEDICATION ADMINISTRATION SERVICES.~~

39 ~~8. NONEMERGENCY TRANSPORTATION.~~

40 ~~B. THE ADMINISTRATION SHALL ESTABLISH SERVICE DESCRIPTIONS AND~~
41 ~~SCOPE AND STAFFING STANDARDS THROUGH RULE OR POLICY.~~

42 ~~C. HOME AND COMMUNITY-BASED SERVICES MAY BE DELIVERED IN ANY~~
43 ~~RESIDENTIAL SETTING AUTHORIZED UNDER STATE LAW WHEN AN INDIVIDUAL IS~~
44 ~~SUBJECT TO A VALID COURT ORDER, GUARDIANSHIP OR INVOLUNTARY TREATMENT~~
45 ~~AUTHORITY PURSUANT TO THIS TITLE, IF:~~

~~1 1. MEDICAID REIMBURSEMENT UNDER THIS ARTICLE IS LIMITED TO COVERED
2 HOME AND COMMUNITY-BASED SERVICES AND DOES NOT INCLUDE ROOM, BOARD,
3 SUPERVISION FOR CUSTODY OR ENFORCEMENT OF COURT ORDERS.~~

~~2. THE INDIVIDUAL IS AFFORDED PERIODIC REVIEW AND PLANNING FOR
TRANSITION TO A LESS RESTRICTIVE SETTING WHEN CLINICALLY APPROPRIATE.~~

~~6 D. THE ADMINISTRATION MAY ADD, MODIFY OR COMBINE SERVICES~~
~~7 CONSISTENT WITH FEDERAL APPROVAL AND LEGISLATIVE INTENT.~~

8 ~~36-2979.04. Program capacity; enrollment factors; emergency~~
9 ~~placements~~

~~A. SUBJECT TO THE APPROVAL OF THE CENTERS FOR MEDICARE AND MEDICAID SERVICES, THE ADMINISTRATION SHALL IMPLEMENT THE PROGRAM FOR UP TO TWO HUNDRED FIFTY MEMBERS STATEWIDE, INCLUDING TEMPORARY EMERGENCY PLACEMENTS. THE ADMINISTRATION MAY INCREASE ENROLLMENT ABOVE TWO HUNDRED FIFTY MEMBERS IF THE ADMINISTRATION DEMONSTRATES, THROUGH DATA REPORTED PURSUANT TO SECTION 36-2979.06, REDUCED UTILIZATION OF HIGH-COST SERVICES OR COST AVOIDANCE IN STATE-FUNDED SYSTEMS, SUBJECT TO REVIEW BY THE JOINT LEGISLATIVE BUDGET COMMITTEE. ENROLLMENT MAY NOT EXCEED ONE THOUSAND MEMBERS UNLESS AUTHORIZED BY THE LEGISLATURE.~~

~~19 B. WHEN CAPACITY IS LIMITED, THE ADMINISTRATION SHALL ASSIGN~~
~~20 AVAILABLE SLOTS BASED ON THE FOLLOWING:~~

21 ~~1. INDIVIDUALS UNDER COURT-ORDERED TREATMENT.~~

~~2. INDIVIDUALS WITH LEGAL GUARDIANSHIP DUE TO PSYCHIATRIC INCAPACITY.~~

~~3. INDIVIDUALS DISCHARGED FROM A JAIL OR PRISON, THE STATE HOSPITAL
OR A BEHAVIORAL HEALTH RESIDENTIAL FACILITY.~~

26 ~~4. INDIVIDUALS WHO ARE HOMELESS OR AT IMMINENT RISK OF~~
27 ~~HOMELESSNESS.~~

~~5. INDIVIDUALS WITH REPEATED CRISIS EPISODES, PSYCHIATRIC
HOSPITALIZATIONS OR PUBLIC SAFETY INVOLVEMENT.~~

~~30 6. INDIVIDUALS PRESENTING SIGNIFICANT SAFETY RISKS DUE TO~~
~~31 PSYCHIATRIC SYMPTOMS.~~

~~7. INDIVIDUALS WITH HIGH-INTENSITY OR COMPLEX PSYCHOTROPIC
MEDICATION REGIMENS REQUIRING ENHANCED MONITORING TO ENSURE ADHERENCE AND
TO IDENTIFY POTENTIAL ADVERSE EFFECTS.~~

~~C. A COURT MAY RECOMMEND PARTICIPATION IN THE PROGRAM BUT MAY NOT
COMPEL THE ADMINISTRATION TO EXCEED THE ENROLLMENT CAP.~~

~~D. THE ADMINISTRATION MAY RESERVE A PORTION OF AVAILABLE CAPACITY
FOR EMERGENCY OR PRIORITY PLACEMENTS.~~

39 ~~36-2979.05. Provider requirements; enhanced reimbursement~~
40 ~~rates~~

~~41 A. PROVIDERS SHALL DOCUMENT BEHAVIORAL INTERVENTIONS, CRISIS~~
~~42 SUPPORTS AND STAFFING ADJUSTMENTS BEFORE INITIATING DISCHARGE.~~

~~B. A PROVIDER THAT RECEIVES REIMBURSEMENT UNDER THE PROGRAM SHALL
IMPLEMENT EVICTION-PREVENTION PROTOCOLS AND OBTAIN REGIONAL BEHAVIORAL
HEALTH AUTHORITY APPROVAL BEFORE ISSUING NONEMERGENCY NOTICES TO VACATE.~~

~~C. THE ADMINISTRATION MAY ADOPT ENHANCED REIMBURSEMENT RATES FOR HIGH-ACUITY SERIOUSLY MENTALLY ILL INDIVIDUALS WHO ARE RECEIVING HOME AND COMMUNITY-BASED SERVICES.~~

~~D. THIS SECTION DOES NOT LIMIT THE ADMINISTRATION'S AUTHORITY TO ENSURE THE HEALTH AND SAFETY OF PARTICIPANTS.~~

~~E. THIS SECTION DOES NOT REQUIRE A PROVIDER TO CONTINUE SERVICES WHEN IMMEDIATE AND DOCUMENTED RISKS TO HEALTH OR SAFETY CANNOT BE MITIGATED THROUGH REASONABLE CLINICAL INTERVENTIONS.]~~

36-2979.03. Enrollment in pilot program; application; delegation of clinical eligibility; waitlist management

[A. AN INDIVIDUAL WHO SEEKS TO ENROLL IN THE PILOT PROGRAM SHALL APPLY THROUGH THE INDIVIDUAL'S CONTRACTOR. AFTER DETERMINING THE INDIVIDUAL MEETS THE SMI ENHANCED LEVEL OF CARE CRITERIA, THE CONTRACTOR SHALL FORWARD THE APPLICATION TO THE ADMINISTRATION FOR FINANCIAL ELIGIBILITY DETERMINATION PURSUANT TO SECTION 36-2979.02, SUBSECTION B. IF THE INDIVIDUAL IS NOT YET ENROLLED WITH A CONTRACTOR, THE INDIVIDUAL MAY APPLY THROUGH THE ADMINISTRATION.]

B. SUBJECT TO THE APPROVAL OF THE CENTERS FOR MEDICARE AND MEDICAID SERVICES, AVAILABLE APPROPRIATIONS AND THE ENROLLMENT CAP PRESCRIBED IN SECTION 36-2979.01, SUBSECTION F, THE ADMINISTRATION MAY DELEGATE TO CONTRACTORS THE AUTHORITY TO DETERMINE WHETHER AN APPLICANT MEETS THE SMI ENHANCED LEVEL OF CARE CRITERIA ADOPTED BY THE ADMINISTRATION PURSUANT TO SECTION 36-2979.02, SUBSECTION E AND TO MANAGE PILOT PROGRAM WAITLISTS, ENROLLMENT DECISIONS AND CAPACITY PRIORITIZATION FOR INDIVIDUALS WHO HAVE BEEN DETERMINED FINANCIALLY ELIGIBLE BY THE ADMINISTRATION.

C. THE ADMINISTRATION OR A CONTRACTOR EXERCISING DELEGATED AUTHORITY UNDER THIS SECTION SHALL:

1. APPLY THE ELIGIBILITY CRITERIA ADOPTED BY THE ADMINISTRATION PURSUANT TO SECTION 36-2979.02.

2. IMPLEMENT AND MAINTAIN ONE OR MORE WAITLISTS, THE TOTAL OF WHICH MAY NOT EXCEED ONE HUNDRED APPLICANTS, FOR APPLICANTS FOR WHOM ELIGIBILITY HAS BEEN DETERMINED.

3. IF THE WAITLIST REACHES ONE HUNDRED APPLICANTS, IMPLEMENT AN INTEREST LIST FOR ADDITIONAL APPLICANTS AND EMPLOY PHASED ENROLLMENT OR OTHER UTILIZATION CONTROLS AS NECESSARY TO ENSURE THAT THE ENROLLMENT CAP PRESCRIBED IN SECTION 36-2979.01, SUBSECTION F IS NOT EXCEEDED.

4. ASSIGN AVAILABLE PILOT PROGRAM CAPACITY BY APPLYING THE PRIORITY FACTORS PRESCRIBED IN SUBSECTION D OF THIS SECTION.

5. REPORT ENROLLMENT DATA TO THE ADMINISTRATION IN THE FORM AND FREQUENCY REQUIRED BY THE ADMINISTRATION BY POLICY.

D. IF THE NUMBER OF FINANCIALLY AND CLINICALLY ELIGIBLE INDIVIDUALS EXCEEDS AVAILABLE PILOT PROGRAM CAPACITY, THE CONTRACTOR SHALL ASSIGN AVAILABLE SLOTS BASED ON THE HIGHEST CLINICAL ACUITY AND FUNCTIONAL IMPAIRMENT DEFINED BY A METHODOLOGY THAT TAKES INTO ACCOUNT THE FOLLOWING PRIORITY FACTORS:

1 1. INDIVIDUALS UNDER COURT-ORDERED TREATMENT.
2 2. INDIVIDUALS WITH LEGAL GUARDIANSHIP DUE TO PSYCHIATRIC
3 INCAPACITY.
4 3. INDIVIDUALS RECENTLY DISCHARGED FROM JAIL OR PRISON, THE STATE
5 HOSPITAL OR A BEHAVIORAL HEALTH RESIDENTIAL FACILITY.
6 4. INDIVIDUALS WHO ARE HOMELESS OR AT IMMINENT RISK OF
7 HOMELESSNESS.
8 5. INDIVIDUALS WITH REPEATED CRISIS EPISODES, PSYCHIATRIC
9 HOSPITALIZATIONS OR PUBLIC SAFETY INVOLVEMENT.
10 6. INDIVIDUALS PRESENTING SIGNIFICANT SAFETY RISKS DUE TO
11 PSYCHIATRIC SYMPTOMS.
12 7. INDIVIDUALS WITH HIGH-INTENSITY OR COMPLEX PSYCHOTROPIC
13 MEDICATION REGIMENS REQUIRING ENHANCED MONITORING TO ENSURE ADHERENCE AND
14 TO IDENTIFY POTENTIAL ADVERSE EFFECTS.
15 E. THE ADMINISTRATION SHALL RETAIN AUTHORITY TO REVIEW AND REVERSE
16 CONTRACTOR CLINICAL ELIGIBILITY AND ENROLLMENT DETERMINATIONS AND TO AUDIT
17 CONTRACTOR COMPLIANCE WITH ELIGIBILITY AND ENROLLMENT REQUIREMENTS.
18 F. THE ADMINISTRATION MAY RESERVE A PORTION OF AVAILABLE ENROLLMENT
19 CAPACITY FOR EMERGENCY OR PRIORITY PLACEMENTS.]
20 36-2979.04. Covered services
21 [A. SUBJECT TO THE APPROVAL OF THE CENTERS FOR MEDICARE AND
22 MEDICAID SERVICES, AVAILABLE APPROPRIATIONS AND THE ENROLLMENT CAP
23 PRESCRIBED IN SECTION 36-2979.01, SUBSECTION F, EACH CONTRACTOR SHALL
24 PROVIDE ENROLLED MEMBERS WITH THE FOLLOWING SERVICES:
25 1. ALL SERVICES THAT THE CONTRACTOR IS REQUIRED TO PROVIDE TO
26 SERIOUSLY MENTALLY ILL INDIVIDUALS UNDER THE CONTRACTOR'S AGREEMENT WITH
27 THE ADMINISTRATION PURSUANT TO SECTIONS 36-3407 AND 36-3410, INCLUDING
28 BEHAVIORAL HEALTH SERVICES, PHYSICAL HEALTH SERVICES, CRISIS SERVICES,
29 CASE MANAGEMENT AND PEER SUPPORT SERVICES.
30 2. ENHANCED RESIDENTIAL TREATMENT SERVICES.
31 B. THE CONTRACTOR SHALL CONDUCT, AT MINIMUM, AN ANNUAL REVIEW OF
32 THE ONGOING MEDICAL NECESSITY OF SERVICES FOR EACH MEMBER AND SHALL ENGAGE
33 WITH THE MEMBER TO PLAN FOR TRANSITION TO A LESS RESTRICTIVE SETTING WHEN
34 CLINICALLY APPROPRIATE.
35 C. THE ADMINISTRATION MAY ADD, MODIFY OR COMBINE SERVICES
36 CONSISTENT WITH FEDERAL APPROVAL AND LEGISLATIVE INTENT, SUBJECT TO THE
37 APPROVAL OF MONIES.]
38 36-2979.05. Service plan; assessment
39 [A. THE CONTRACTOR SHALL DEVELOP AND MAINTAIN A WRITTEN SERVICE
40 PLAN FOR EACH MEMBER. THE SERVICE PLAN MUST:
41 1. BE BASED ON A COMPREHENSIVE ASSESSMENT OF THE MEMBER'S
42 BEHAVIORAL HEALTH, FUNCTIONAL AND SAFETY NEEDS.
43 2. IDENTIFY THE SPECIFIC SERVICES AND SUPPORT TO BE PROVIDED AND
44 THE SETTINGS IN WHICH THE SERVICES AND SUPPORT WILL BE PROVIDED.
45 3. INCLUDE GOALS FOR RECOVERY, STABILITY AND, IF APPROPRIATE,
46 TRANSITION TO LESS RESTRICTIVE SETTINGS.

1 4. BE UPDATED AT LEAST ANNUALLY OR MORE FREQUENTLY IF THE MEMBER'S
2 NEEDS CHANGE.

3 B. THE MEMBER AND, IF APPLICABLE, THE MEMBER'S GUARDIAN OR
4 AUTHORIZED REPRESENTATIVE SHALL PARTICIPATE IN THE DEVELOPMENT AND
5 REVISION OF THE SERVICE PLAN.]

6 36-2979.06. Enhanced residential treatment services; provider
7 requirements; transition and discharge planning
8 protocols; reimbursement

9 [A. PROVIDERS OF ENHANCED RESIDENTIAL TREATMENT SERVICES SHALL
10 DOCUMENT BEHAVIORAL INTERVENTIONS, CRISIS SUPPORT AND STAFFING ADJUSTMENTS
11 BEFORE INITIATING THE DISCHARGE OF ANY MEMBER.

12 B. A PROVIDER OF ENHANCED RESIDENTIAL TREATMENT SERVICES SHALL
13 IMPLEMENT TRANSITION AND DISCHARGE PLANNING PROTOCOLS AND SHALL OBTAIN
14 APPROVAL FROM THE CONTRACTOR BEFORE ISSUING TO A MEMBER A NONEMERGENCY
15 NOTICE TO VACATE.

16 C. THE ADMINISTRATION MAY ADOPT ENHANCED REIMBURSEMENT RATES FOR
17 ENHANCED RESIDENTIAL TREATMENT SERVICES PROVIDED TO MEMBERS UNDER THIS
18 ARTICLE. THE ADMINISTRATION SHALL INCORPORATE REIMBURSEMENT RATES AND
19 METHODOLOGIES INTO CONTRACTOR AGREEMENTS PURSUANT TO SECTION 36-3412.

20 D. THIS SECTION DOES NOT:

21 1. LIMIT THE ADMINISTRATION'S AUTHORITY TO ENSURE THE HEALTH AND
22 SAFETY OF MEMBERS.

23 2. REQUIRE A PROVIDER TO CONTINUE SERVICES WHEN IMMEDIATE AND
24 DOCUMENTED RISKS TO HEALTH OR SAFETY CANNOT BE MITIGATED THROUGH
25 REASONABLE CLINICAL INTERVENTIONS.

26 3. REQUIRE A CONTRACTOR TO CONTINUE PAYING FOR SERVICES THAT ARE
27 NOT MEDICALLY NECESSARY.]

28 36-2979.07. Grievance and appeal process

29 [A. A MEMBER OR APPLICANT AGGRIEVED BY AN ACTION OF THE
30 ADMINISTRATION OR A CONTRACTOR RELATING TO ELIGIBILITY, ENROLLMENT OR
31 SERVICES UNDER THIS ARTICLE HAS THE RIGHT TO FILE A GRIEVANCE OR APPEAL
32 PURSUANT TO SECTION 36-3413 AND, IF APPLICABLE, TO A HEARING PURSUANT TO
33 RULES ADOPTED PURSUANT TO SECTION 36-2903.01, SUBSECTION B.

34 B. THE CONTRACTOR'S GRIEVANCE AND APPEAL PROCESS REQUIRED UNDER
35 SECTION 36-3413 APPLIES TO MATTERS ARISING UNDER THIS ARTICLE, INCLUDING
36 DENIALS, REDUCTIONS OR TERMINATIONS OF ENHANCED RESIDENTIAL TREATMENT
37 SERVICES AND CONTRACTOR CLINICAL ELIGIBILITY DETERMINATIONS.

38 C. APPEALS OF FINANCIAL ELIGIBILITY DETERMINATIONS MADE BY THE
39 ADMINISTRATION PURSUANT TO SECTION 36-2979.02, SUBSECTION B SHALL BE HEARD
40 PURSUANT TO RULES ADOPTED PURSUANT TO SECTION 36-2903.01, SUBSECTION B.]

41 ~~[36-2979.06.]~~ ~~[36-2979.08.]~~ Annual report

42 BEGINNING ONE YEAR AFTER [PILOT] PROGRAM IMPLEMENTATION AND EACH
43 YEAR THEREAFTER [FOR THE DURATION OF THE PILOT PROGRAM], THE
44 ADMINISTRATION SHALL SUBMIT A REPORT TO THE GOVERNOR, THE PRESIDENT OF THE
45 SENATE AND THE SPEAKER OF THE HOUSE OF REPRESENTATIVES THAT INCLUDES:

1 1. THE NUMBER OF ~~[ENROLLED]~~ MEMBERS AND THE NUMBER OF INDIVIDUALS
2 WHO ARE ON THE WAITLIST ~~[AND THE NUMBER OF INDIVIDUALS WHO ARE ON THE~~
3 ~~INTEREST LIST FOR SERVICES AND WHO HAVE NOT YET BEEN ENROLLED, IF~~
4 ~~APPLICABLE]~~.

5 2. THE PERCENTAGE OF MEMBERS WHO ARE UNDER COURT-ORDERED TREATMENT,
6 GUARDIANSHIP OR CRIMINAL JUSTICE SUPERVISION.

7 3. HOUSING STABILITY OUTCOMES FOR MEMBERS.

8 4. HOSPITALIZATION, CRISIS SERVICE, JAIL, EMERGENCY DEPARTMENT,
9 EVALUATION AGENCY AND SCREENING AGENCY UTILIZATION OF MEMBERS.

10 5. PROVIDER DENIALS AND DISCHARGES AND THE REASONS FOR THE DENIALS
11 AND DISCHARGES OF MEMBERS.

12 6. THE FISCAL IMPACT OF AND ESTIMATED COST AVOIDANCE RELATED TO THE
13 ~~[PILOT] PROGRAM.~~

14 ~~[7. THE NUMBER OF MEMBERS WHO ARE SERVED IN ENHANCED RESIDENTIAL~~
15 ~~TREATMENT FACILITIES.]~~

16 ~~[36-2979.07.]~~ [36-2979.09.] Authority to seek and maintain
17 federal approval

18 A. THE ADMINISTRATION SHALL SEEK AND MAINTAIN ANY NECESSARY FEDERAL
19 APPROVALS AND MAY OPERATE THE ~~[PILOT]~~ PROGRAM PURSUANT TO ONE OR MORE
20 APPROVED MEDICAID STATE PLAN AUTHORITIES, WAIVERS OR DEMONSTRATION
21 AUTHORITIES, INCLUDING THOSE AUTHORIZED UNDER 42 UNITED STATES CODE
22 SECTION 1315 OR ~~[1396r]~~ [OTHER APPLICABLE FEDERAL AUTHORITY].

23 B. IF THE CENTERS FOR MEDICARE AND MEDICAID SERVICES DENIES OR DOES
24 NOT APPROVE A REQUEST NECESSARY TO IMPLEMENT THIS ARTICLE, THE
25 ADMINISTRATION, TO THE EXTENT ALLOWED BY FEDERAL LAW, SHALL CONTINUE TO
26 PURSUE APPROVAL THROUGH MODIFICATIONS, RESUBMISSIONS OR ALTERNATIVE
27 FEDERAL AUTHORITY AND SHALL REPORT THE STATUS OF THESE EFFORTS IN THE
28 SEMIANNUAL IMPLEMENTATION UPDATES REQUIRED PURSUANT TO SECTION 36-2979.01.
29 ~~[THE ADMINISTRATION SHALL OBTAIN LEGISLATIVE APPROVAL FOR MODIFICATIONS OR~~
30 ~~RESUBMISSIONS THAT ARE INCONSISTENT WITH THIS ARTICLE.]~~

31 ~~[C. THE ADMINISTRATION MAY ADOPT RULES TO IMPLEMENT THIS ARTICLE.]~~
32 ~~[C. ON FEDERAL APPROVAL AND SUBJECT TO AVAILABLE APPROPRIATIONS,~~
33 ~~THE ADMINISTRATION MAY IMPLEMENT THIS ARTICLE THROUGH POLICY AND CONTRACT~~
34 ~~REQUIREMENTS PURSUANT TO SECTIONS 36-2903, 36-2904 AND 36-3412.]~~

35 D. THIS ARTICLE DOES NOT EXPAND OR MODIFY STANDARDS FOR INVOLUNTARY
36 TREATMENT UNDER THIS TITLE.

37 36-2979.10. Service plan; assessment

38 [A. SERVICES PROVIDED UNDER THIS ARTICLE SHALL SUPPLEMENT BUT NOT
39 REPLACE ANY OTHER BENEFITS OR SERVICES FOR WHICH A MEMBER IS OTHERWISE
40 ELIGIBLE.]

41 [B. THE ADMINISTRATION SHALL COORDINATE BENEFITS WITH OTHER PAYORS,
42 INCLUDING MEDICARE AND PRIVATE INSURANCE, TO THE EXTENT REQUIRED BY
43 FEDERAL LAW.]

44 [C. A MEMBER IN THE PILOT PROGRAM MAY NOT ALSO BE ENROLLED IN THE
45 ARIZONA LONG-TERM CARE SYSTEM PURSUANT TO ARTICLE 2 OF THIS CHAPTER.]

1 <<Sec. 5. Delayed repeal; condition; notice
2 [A. The seriously mentally ill enhanced residential treatment pilot
3 program established by title 36, chapter 29, article 3.1, Arizona Revised
4 Statutes, as added by this act, is repealed three years after the date
5 that the centers for medicare and medicaid services approve the pilot
6 program.
7 B. The Arizona health care cost containment system administration
8 shall notify the director of the Arizona legislative council in writing of
9 this date.]>>

10 Enroll and engross to conform
11 Amend title to conform

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